SUBJECTIVE: The patient is a 34-year-old female with an established 4-year history of ulcerative pancolitis presenting for clinical re-evaluation. She is currently maintained on oral mesalamine 4.8 g daily. Over the past three weeks, she reports experiencing an acute flare characterized by 6 to 8 loose, watery bowel movements per day containing visible bright red blood and mucus, accompanied by lower abdominal cramping and nocturnal defecation. She denies fever, weight loss, or extraintestinal manifestations such as peripheral arthralgias, uveitis, or erythema nodosum.

PHYSICAL EXAMINATION:

General: Alert, comfortable, in no acute distress.

Vital Signs: Blood pressure 112/68 mmHg, pulse 78 bpm, temperature 36.9 degrees Celsius.

Abdomen: Flat, soft, mild tenderness over the left lower quadrant along the course of the descending and sigmoid colon. No involuntary guarding, rigidity, or peritoneal signs. Bowel sounds hyperactive.

Extremities: No peripheral edema, no skin lesions.

DIAGNOSTIC WORKUP:

Complete Blood Count: WBC 9,800/uL, Hemoglobin 11.4 g/dL, Platelets 420,000/uL (reactive thrombocytosis).

Inflammatory Markers: C-reactive protein (CRP) elevated at 28.4 mg/L (reference < 5.0 mg/L); Fecal calprotectin markedly elevated at 840 ug/g (reference < 50 ug/g).

Stool Microbiological Studies: Clostridioides difficile PCR toxin negative; routine stool enteric bacterial cultures (Salmonella, Shigella, Campylobacter, E. coli O157) negative; Ova and parasites negative.

Flexible Sigmoidoscopy: Performed in clinic without sedation, advanced to 40 cm. Visualized continuous, circumferential mucosal erythema, loss of normal vascular pattern, marked mucosal friability with contact bleeding, and superficial confluent ulcerations corresponding to a Mayo Endoscopic Subscore of 2 (Moderate Ulcerative Colitis).

ASSESSMENT AND TREATMENT PLAN:

1. Acute moderate flare of ulcerative colitis failing mesalamine monotherapy.

2. Initiate oral corticosteroid induction with prednisone 40 mg daily for two weeks, followed by a structured 5 mg per week taper.

3. Advance maintenance therapy to biologic or small molecule therapy. Screen for latent infections with QuantiFERON-TB Gold, Hepatitis B panel, and varicella titers today.

4. Plan initiation of anti-TNF therapy (e.g., infliximab biosimilar) or anti-integrin therapy (vedolizumab) following pre-biologic clearance.